



23/10/2024

Dr Alexander Amory
Cooks Hill Family Practice
Cooks Hill Commercial Centre
235 Darby Street
COOKS HILL NSW 2300

Dear Alexander,

RE: MrStephen COOKE DOB: 27/10/1981
7a Bakeri Circuit, WARABROOK NSW 2304
Consultation date: 23/10/2024

I asked Stephen to come in today as he wanted to discuss some changes which have happened in his life and with the ongoing legal case.

I am aware Stephen has conducted a hair follicle test for D&A on his own volition, results of which I have to hand.

I have never had concerns of Drug and Alcohol misuse as part of Stephen's history. This was evident as well in his report. The positive result for amphetamines would correlate to the prescribed dexamphetamine and lisdexamphetamine which he takes daily.

He is doing well with components of executive dysfunction managed on lisdexamphetamine 30mg and dexamphetamine 5mg, 2 a day as top ups.

I had informed Stephen that I had received a subpoena of his medical records which I will be completing and submitting as requested.

I believe the latest update in his proceedings has been completion of financial settlement and is now focusing on parental agreement on the children and custody/care arrangements.

Stephen has completed his internship with Diamond IT however was unable to complete the certification that was mandated to secure an entry level job as he did not have the required allowance of time to dedicate to study.

This was largely secondary to having to spread himself thin trying to keep abreast of his legal proceedings while also trying to juggle day to day routines and his boys care when he has them.

Stephen is acutely aware of the importance of having a job to be financially stable to not only be self sufficient but also to provide a stable and consistent home for both his boys.

We spoke about the best option in this case which is likely a part time job. This will allow protected time for him to complete further certifications with the end goal of securing a full time job in IT.

Working part time would also give him the flexibility to be available and present for his boys who have always been his priority.

Other components of Stephen's life are reasonably balanced with his personal/health investments, self care, house chores/upkeep and involvement in the community at church.

There continues to be growth with Stephen since his diagnosis and management in being able to navigate and working towards balance in life.

2/...

Re Stephen Cooke Page 2

Despite the ongoing legal and its impact personally, his engagement in regular sessions with me and accountability in maintaining healthy outlets, not indulging in substances including alcohol has seen his overall mental state remain stable with no risk concerns.

I will see him back in December.

Kind regards,

A handwritten signature in black ink, appearing to be 'S. Moisey', written in a cursive style.

Dr Suraiya Moisey BMed FRANZCP

Consultant Psychiatrist

Provider No: 430478WF



RE: **MrStephen COOKE** DOB: 27/10/1981
7a Bakeri Circuit, WARABROOK NSW 2304
Consultation date: 23/10/2024

Dexamfetamine sulfate 5mg - Oral Tablet - Oral

-

5MG

2 A DAY

ePrescribed on 20/09/2024

Sent

Lisdexamfetamine dimesilate 30mg - Oral Capsule - Oral

-

30mg

one a day

ePrescribed on 20/09/2024

Sent

Lisdexamfetamine dimesilate 30mg - Oral Capsule - Oral

-

30MG

ONE A DAY

ePrescribed on 21/06/2024

Sent

Dexamfetamine sulfate 5mg - Oral Tablet - Oral

-

5mg

one a day

ePrescribed on 06/06/2024

Sent

Lisdexamfetamine dimesilate 20mg - Oral Capsule - Oral

-

20MG

ONE A DAY

ePrescribed on 02/05/2024

Sent

2/....

Re Stephen Cooke Page 2

Lisdexamfetamine dimesilate 20mg - Oral Capsule - Oral

-

20MG

ONE A DAY

Prescribed on 02/04/2024

Dexamfetamine sulfate 5mg - Oral Tablet - Oral

-

5MG

1 A DAY

Prescribed on 09/02/2024

Lisdexamfetamine dimesilate 20mg - Oral Capsule - Oral

-

20mg

one a day

Prescribed on 09/02/2024

Lisdexamfetamine dimesilate 20mg - Oral Capsule - Oral

-

20MG

ONE A DAY

Prescribed on 01/12/2023

Dexamfetamine sulfate 5mg - Oral Tablet - Oral

-

5MG

1 A DAY

Prescribed on 02/11/2023

Lisdexamfetamine dimesilate 20mg - Oral Capsule - Oral

-

20MG

1 A DAY

Prescribed on 02/11/2023

Lisdexamfetamine dimesilate 20mg - Oral Capsule - Oral

-

20mg

one a day

Prescribed on 27/09/2023

3/...

Re Stephen Cooke Page 3

Kind regards,

A handwritten signature in black ink, appearing to be 'S. Moisey', written in a cursive style with a long horizontal stroke extending to the left.

Dr Suraiya Moisey BMed FRANZCP

Consultant Psychiatrist

Provider No: 430478WF

25/09/2024
Dr Alexander TONKIN
Newcastle General Practice
62 Denison Street
HAMILTON EAST NSW 2303

Dear Alexander,

RE: MrStephen COOKE DOB: 27/10/1981
7a Bakeri Circuit, WARABROOK NSW 2304
Consultation date: 20/09/2024

This was Stephen's 3-month appointment.

Biological regime is working well on Vyvanse 30mg with dexamphetamine 5 mg 1 tablet in the morning with the option of another either at the tail end or in addition depending on the task at hand.

There continues to be ongoing psychosocial stressors in his life which he had and does always discuss with me.

Stephen had single-handedly cleaned and fixed the house where they're residing, to get it ready for sale which he managed to do successfully.

Since then he has moved into a rental property in Warabrook which is in a nice neighbourhood however the place needs a lot of work which Stephen has been focusing on.

Stephen is doing well in balancing the requirements of house duties alongside the care of his kids when he has them. The other parts of his time is spent on this arduous chaotic legal matter and has also started an internship with an IT company which he is hoping to secure a job with once he completes it.

Court proceedings continue however from my understanding Stephen has developed remarkable capacity to engage lateral thinking and curiosity and is challenging situations when he feels there are discrepancies.

Stephen has demonstrated the ability to juggle responsibilities at hand while also being cognisant about the need to self care, his food, exercise and sleep hygiene.

I will see him back in 3 months time.

Yours sincerely



Dr Suraiya Moisey BMed FRANZCP
Consultant Psychiatrist
Provider No: 430478WF

26/06/2024
Dr Alexander TONKIN
Newcastle General Practice
62 Denison Street
HAMILTON EAST NSW 2303

Dear Alexander,

RE: MrStephen COOKE DOB: 27/10/1981
1/213 Morgan Street, MEREWETHER NSW 2291
Consultation date: 21/06/2024

This was Stephen's 2-month appointment.

He has remained compliant to Vyvanse 20 mg with a top of 5 mg Dex as advised at the tail end

Since have seen Stephen last, there has been far more responsibilities on his plate as he continues to juggle the complexity of the legal issues, being a dad to his 2 boys who is now has reduce access to as per interim orders where the time he has with them he dedicates solely to being a father, maintenance of the household, and more recently studying to up skill in courses in an effort to find a job.

Stephen is acutely aware that in managing multitude of responsibilities the importance of routine from his sleep-wake cycle, dietary regulation and exercise which of late has been rollerskating and running.

He has been eating a lot better.
Stephen can appreciate the impact of healthy dopamine.

The stress of his marriage and his family break up is most certainly a factor however it has not resulted in any concerns with depressive symptoms.

Stephen certainly demonstrates resilience and knowing what is within his control and that his foremost importance lies with the wellbeing of his children.

Stephen's focus not only for the now but also in the future and the plan he needs to make to be able to provide the self-harm for his family.

With the increase in responsibilities on his plate I have suggested an increase in his Vyvanse to 30 mg for further coverage and a further dexamphetamine particularly as he studies later in the evening

I will see Stephen back in 2 months time

Yours sincerely



Dr Suraiya Moisey BMed FRANZCP
Consultant Psychiatrist
Provider No: 430478WF



08/05/2024
Dr Alexander TONKIN
Newcastle General Practice
62 Denison Street
HAMILTON EAST NSW 2303

Dear Alexander,

RE: MrStephen COOKE DOB: 27/10/1981
1/213 Morgan Street, MEREWETHER NSW 2291
Consultation date: 07/05/2024

It has been a stressful few months for Stephen.

He has a legal team who is supporting him through the complexity of the children's care/custody arrangement and the property.

It is a complicated web however he seems to be navigating this with precision and caution, not leaving a stone unturned.

Stephen seems to have picked the solicitor up and challenged her when there are inconsistencies and lack of transparency in their decisions however my understanding is that Stephen doesn't have confidence and faith in their service and is often having to re evaluate and reassess their work which I can only imagine is emotionally exhausting.

Given the sensitive nature of his personal matter, I have suggested it would be best to source a team in Sydney away from the familiarity in Newcastle.

He is trying his best to juggle the legal matter, finding a job, getting the house ready for sale (as part of the court orders which dictates house has to be sold by July 11th or it goes to auction) and being the present, caring father he has always been with his boys.

Medications have been helpful particularly now when the demands of executive functioning is put to the test. Stephen is doing his best within his capacity while also recognising the importance of his health and wellbeing.

Aside from the stability with medications, he is eating better, does not consume any substance that can alter his perception and is invested in his support network of friends and family.

His interest is only in what's best for his boys.

I have advised him, recognising the overwhelm on his plate that he needs to currently prioritise his decision on a legal team where he has trust in their competency and selling of the house.

These tasks are monumental and sufficient of an investment now to allow his emotional cup to be invested solely with the boys.

2/...

Re Stephen Cooke Page 2

I will see him back in 6 weeks.

Kind regards,

A handwritten signature in black ink, appearing to be 'S. Moisey', written in a cursive style with a long horizontal stroke extending to the left.

Dr Suraiya Moisey BMed FRANZCP

Consultant Psychiatrist

Provider No: 430478WF

14/02/2024
Dr Alexander TONKIN
Newcastle General Practice
62 Denison Street
HAMILTON EAST NSW 2303

Dear Alexander,

RE: MrStephen COOKE DOB: 27/10/1981
1/213 Morgan Street, MEREWETHER NSW 2291
Consultation date: 09/02/2024

This was Stephen's 3 month appointment.

There has been a lot of stress, personal and familial on his plate which has seen him battle waves of emotions, even grief from having his family unit and children taken away.

Stephen purports that Heather's leaving and removal of furniture/belongings was done in haste and suddenly which blind sided him and the boys.

Sadly this happened the same day that she participated with Stephen and the boys to put up their Christmas tree. Stephen has been devastated at the impact it has had on his boys whom he was the primary care giver for.

There has been since then a whirlwind of professional bodies involvement from a legal standing which he has participated in, hoping he would have 50/50 custody but relented to the current interim arrangement when he was threatened by her side of a vacuum process.

When he is not with the boys, he has been working towards job applications, networking and polishing up components of his training in IT which would be required for these jobs.
He now has his resume on Seek.

Stephen has also been looking after his health recognising that while it would be easy to succumb to the current psychological upheaval, he has been eating a lot better, exercising, keeping alcohol to a minimum and sleeping reasonably well.

Stephen has been able to invest in the balance of professional and personal life reasonably well given his current predicament of an acrimonious divorce.

Since being medicated, with vyvanse 20mg in the day and dexamphetamine 5mg as a top up, he has continued to notice and utilise the benefits in executive functioning currently needed to navigate the complexity of documents, conversations, proceedings, meetings, mediation and preparation for upcoming court well.

His perspective in being able to focus, maintain attention with clarity has allowed him to initiate, persevere and complete necessary tasks based on level of priority in a time efficient manner.

The court date is set for early April. Stephen has the support of his legal team, family in Canada, and a close network of friends here.

2/...

Re Stephen Cook Page 2

No change to medications.

I will see him back in 3 months time.

Kind regards,

A handwritten signature in black ink, appearing to be 'S. Moisey', written in a cursive style.

Dr Suraiya Moisey BMed FRANZCP

Consultant Psychiatrist

Provider No: 430478WF

07/12/2023

Dr Alexander TONKIN

Newcastle General Practice

62 Denison Street

HAMILTON EAST NSW 2303

Dear Alexander,

RE: MrStephen COOKE DOB: 27/10/1981

1/213 Morgan Street, MEREWETHER NSW 2291

Consultation date: 01/12/2023

This was Stephen's 1 month appointment.

Unfortunately there has been a major eruption in his personal life.

From Stephen's perspective, the situation has escalated beyond his belief. Allegedly, Heather had wanted Stephen to sign on the dotted line to allow her financial access to buy into the practice. When he had refused to agree, she had tried to get him to sign the house as collateral. Fortunately he did his own investigations and quite quickly realised doing so would see him destitute. Again when he refused, several days later she transferred all the money out of the joint account (from properties they co own) into her personal account.

In disbelief, fortunately, Stephen was able to act accordingly and sought help from a lawyer and from a financial support institution and has now claimed financial abuse.

This has been the trigger to further fracture his already devolving marriage. It would seem that Stephen and Heather will be going through separation and then divorce which is most certainly going to be acrimonious.

The boys are privy to this disharmony and Christian who already struggles with his mental health is not doing well. Allegedly, Heather has been playing mind games with them. Her inconsistency in parenting and her intolerance has certainly heightened. She has threatened to take the kids away from him and move out. Stephen suspects that she has another man but she has not given him confirmation. He does not trust her and now in her presence feels disgusted, used and abused. Up until a couple of weeks ago, Stephen was still trying to make the marriage work but at this point is broken and destroyed.

He did go through a few days of breaking down but has been able to pull himself together and continues as he has always done by focusing on the kids wellbeing. Stephen is confident that he has been the stable parental figure and has provided consistency for both his boys. Stephen will fight tooth and nail to have majority custody but is under no illusion that this will be an easy battle either.

The medication has been helpful in keeping his perspective and focus. Stephen certainly does not demonstrate any symptoms of an Axis I mood disorder despite the current adversities.

I will check in on Stephen on a monthly basis.

Yours sincerely



Dr Suraiya Moisey BMed FRANZCP

Consultant Psychiatrist - Provider No: 430478WF

08/11/2023
Dr Alexander TONKIN
Newcastle General Practice
62 Denison Street
HAMILTON EAST NSW 2303

Dear Alexander,

RE: MrStephen COOKE DOB: 27/10/1981
1/213 Morgan Street, MEREWETHER NSW 2291
Consultation date: 02/11/2023

This was Stephen's 6-week appointment.

Vyvanse 20 mg has been going well.
He takes it at 9:00 in the morning and at last of 5 PM.

The only negatives are dry mouth.
There has been an appetite reduction however he is now enjoying process of food as this task is now manageable as part of day-to-day responsibilities.
He has also noticed reduced alcohol consumption as it does not have the same affect any longer.

Stephen only complaint, is that the duration is short lived for how long his days are which often starts at 7 AM and does not finish till 8 PM when the kids go down.
He has tried bringing the vyvanse forward but it results in a compromise to the drop off time.
Not having coverage for the later part is a noticeable struggle when it comes to children related responsibilities.

I have suggested trialing dexamphetamine 5 mg first thing in the morning and then delaying the Vyvanse till mid morning.
I am hoping this will provide him with the required 12 hours.

He has noticed an improvement in clarity, calmness, and ability to prioritise and manage time. Stephen also reports that he has been clearing backlog of tasks that for so long he has procrastinated. He has been thinking several steps ahead which he was amazed by.

Heather has also noticed an improvement in his executive functioning and his ability to multitask effectively.

Unfortunately his hobby has now been pushed to the nighttime.
It has somewhat compromised going to bed at a reasonable time which I have reminded him is a bad habit to continue as it will affect the efficacy of the stimulants.

Unfortunately his relationship and intimacy is still an issue.
Heather is reluctant to see a marriage counsellor and any advances made by Steven to discuss this matter is met with hostility.
Stephen feels a sexless marriage is not one he can stay in.

He also feels at a disadvantage to her and certainly not an equal.

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Pursuing this further would only be futile. I have suggested his best course is to invest in himself and his self worth. This would include actively looking for work, social development and hobbies.

I will see him back in a months time.

Yours sincerely

A handwritten signature in black ink, appearing to be 'S. Moisey', written in a cursive style with a long horizontal stroke extending to the left.

Dr Suraiya Moisey BMed FRANZCP

Consultant Psychiatrist

Provider No: 430478WF

10/10/2023
Dr Alexander TONKIN
Newcastle General Practice
62 Denison Street
HAMILTON EAST NSW 2303

Dear Alexander,

RE: MrStephen COOKE DOB: 27/10/1981
1/213 Morgan Street, MEREWETHER NSW 2291

Thank you for a referral for second opinion and ongoing management.

Stephen is a 41-year-old married man to Heather Cook, Specialist cardiologist for the last 10 years. They have 2 boys aged 9 diagnosed with ADHD and 6 1/2 was currently a stay home dad. His family and friends are back in Canada. Stephen and Heather have been back living in Australia for the last 10 years.

Stephen's passion is IT however he never pursued this as a career giving me the impression it was secondary to a sacrifice and failed opportunities.

Stephen came upon encouragement by Heather who suspect he has ADHD. Stephen's symptoms of struggle are very similar to his oldest son. Stephen has also done some reading and would certainly resonate with the diagnosis.

Some of the key elements which he resonates with include

1. hyperfocus on IT. This can be hours to the exclusion of other activities even eating and drinking. It is often to the irritation of his family.

The current dispute is costing him his marriage. His over indulgence and unnecessary investment with needing state-of-the-art IT system.

2. Poor short-term memory.

3. Forgetfulness and ease in misplacing everything.

4. Time blindness and tardiness.

5. If disinterested no matter how much effort he puts in his struggles to keep himself focused and motivated.

He has no known past psychiatric history.

Stephen has no known medical comorbidities either.

Stephen tried weed briefly in University but has never had an issue with substance abuse. Alcohol fidgetiness since he was 14 and became a reliance to manage social anxiety. At 1 point it became daily, drinking 2 to 3 glasses of wine. This is no longer a concern. No issues with caffeine or sugar misuse.

Stephen grew up in Toronto Canada. He is the middle child of 3.

Stephen describes dad as an anxious man and suspicious that he may have ADHD himself. On the same token, he describes dad as a successful man in his business ventures and I suspect the role model Stephen placed on a pedestal. Dad however had a temper, he did not see gray, was set in his ways. Stephen found hard but fair. I wonder if Steven has compensated and normalised but will come across as an abusive authority figure. Stephen often defaulted to dad for reassurance and remained thirsty for validation. I suspect he never felt good enough.

2/...

Stephen is uncertain if mum has any mental health issues. He describes her as permissive and passive. "She was nice, she did all the meals" interestingly when it comes to discipline and being apparent she deflected to dad.

I also suspect that Stephen was the classic middle child with a passive personality and he never lived up to his family's expectations.

For purposes of inclusion or exclusion of ADHD the rest of the first and second assessment focused on identifying clinical criteria.

Hyperactivity is less obvious. He does fidget eliciting either watching TV or doing an activity that he is disinterested in. There is a tendency to stim by tapping his fingers to his thumb and repeating.

Inattention is a significant struggle unless it is IT but this too can see him fall down the rabbit hole.

Impulsivity is another component of struggle particularly for anything computers and IT. Stephen enjoys the hire of pursuing an investing despite problems. He perseveres with the goal being the completion and a sense of accomplishment. Stephen enjoys gaming and other technology based outlets but often feels guilty when he indulges. I suspect this is from him feeling he is not contributing being a stay-at-home dad and therefore not entitled to rest and relaxation.

Sensory overload to the point of frustration is often associated with his kids screaming particularly when he is focused with IT related tasks.

Rejection sensitivity dysphoria is a major component but only when it comes to Heather. Again I suspect this is due to a power imbalance as it does not seem to be present in any other interpersonal relationships. The RSD observed with Heather is felt as a visceral sensation. He often seeks reassurance from Heather whom I feel he puts on a pedestal the very same way he had dad. Heather is seen as the voice of reason and stability however deflecting to her with the relationship down further.

His head is described as busy, constantly thinking and moves a lot faster than his speech.

Executive functioning is poor every stage from planning to execution is a stuckness.

Short-term memory is poor especially if material is present in words or verbally. The concept of out of sight out of mind certainly resonates.

Tardiness is another struggle as his poor time management with due to a tendency to underestimate at every turn.

He remembers himself being hopeless at school. Stephen was assessed as having some form of a learning difficulty and hearing impairment. I wonder if that was an incorrect label for what may have been ADHD. Even with 1-1 support he struggled academically.

In year 8, Steven found IT which started the birth of his passion. He sees ideas and invaluable in practical commodity and everyday life. Stephen did well in computer sciences subject but only when it was hands-on but not when it was theoretical and reading-based.

Unfortunately in early high school, Stephen started smoking and then drinking where he where he started to gravitate towards likeminded individuals. I suspect substances became a Band-Aid for social development and to a degree has featured as pockets in his adult life.

Stephen briefly attended college but did not do well and has never pursued tertiary education again.

He has had a reasonable working experience. Initially work was about bringing in money and was all that he could do without having a degree. He then worked for 10 years in the family business which was insurance based almost without choice. While he did not struggle it was not interesting. Stephen has also done multiple other jobs when in Canada but mostly administrative, low-grade roles.

He tried to pursue an IT course online when he was in Australia earlier in the piece was unable to sustain the course as he was struggling studying with looking after a newborn baby.

The last time he interviewed for a job was with NAB a few years ago. This seemed interested in him however he did not have some of the computing experience that was required. Since the interview, Steven has been teaching himself the courses and is confident if he goes for another interview he will be successful.

In social situations, Stephen finds it difficult to look at people when conversing as he finds faces distracting. Stephen also struggles to articulate a narrative coherently. He needs to consciously focus on how to construct sentences as his head is much faster than his speech and the entire process is overwhelming. He is therefore better at impromptu banter and with Stephen otherwise struggles to stay present, attentive and focused on conversations. He is often been reprimanded for dreaming, getting lost in content going on tangents and derailing. In general his emotional battery is short.

When he comes to chores which is the essentially the largest part of his day to day life, Stephen believes that he gets them done to the best of his abilities. However however has often remarked that it is not good enough as tasks are half done or done incorrectly.

She has also commented on his poor time management, inefficient use of time, poor ability to prioritise tasks and effectively work through them due to the ease of distractibility and his inability to see a clear pathway from A to B.

I also suspect Heather struggles to step back and put herself in his shoes as she would come across as a high achiever and cannot understand inefficiency.

Stephen would certainly admit that he struggles with process orientated tasks such as groceries, meal plan and prepping and cooking. In my opinion the role of house management and maintenance of routine with the kids is a much bigger feat than a professional job but clearly one that he struggles with.

Heather and Stephen are going through a tough time as their marriage is on the rocks. He does not feel good enough and is a burden. Intimacy ended as the oldest child started to exhibit ADHD symptoms and then acquired the diagnosis. Heather has also struggled with accepting the diagnosis and perceives it as parenting failure.

Stephen is hoping to have his dysfunctions and disability recognised and managed which he feels is one large aspect in salvaging their relationship which I suspect is an over simplification. Stephen however reassures me that he is not naïve and knows that there are other issues much bigger than ADHD symptoms and that they would need marital counselling.

On Axis I assessment anxiety is a diagnosis that Stephen is certainly familiar with and not only parallel but secondary to ADHD. I suspect there is underlying anankastic traits and OCPD.

He raised OCD as a possible cluster of symptoms that he may struggle with drawing to my attention to checking behaviour around safety. There is a ritual to it the act of which provides relief but it seems to be exclusive to keeping the children in check and generalised checking of electrical items/devices before leaving the house.

There is no pervasive themes of major depression or any other mood disorder at hand.

In my initial clinical impression, Stephen's primary diagnosis is attention deficit disorder, inattentive subtype. He also struggles with secondary anxiety on a background of cluster C personality traits.

1. I discussed my formulation with him drawing in examples from the history provided.
2. have commenced him on Vyvanse 20 mg giving him information about the pharmacology of the long acting, the benefits to be expected and potential side effects.
3. last year fabricated the responsibility onto him to ensure optimal dietary choices, hydration, some form of exercise and sleep hygiene.
4. psychosocial recovery is the aim with hopefully executive functioning stability with medications. It is about finding his work, purpose and sense of being a man of the house. I have encouraged him to move forward with looking for work as this will certainly provide a better balance of work and home life and help with his confidence and self worth.
5. He is to call me in a week to discuss symptom improvement before further changes are made.
6. I will see him back in a months time.

Yours sincerely

A handwritten signature in black ink, appearing to be 'Dr Suraiya Moisey', written in a cursive style.

Dr Suraiya Moisey BMed FRANZCP
Consultant Psychiatrist
Provider No: 430478WF